

Social Determinants of Health Screening Assessment

MCG Health
Chronic Care
28th Edition

CCG: C-1139 (CCG)
[Link to Codes](#)

- Assessment
- Quality Measures
- Plan of Care
- Evidence Summary
- References
- Codes

Assessment

Note: This assessment is intended to be the first step for screening for social determinants of health needs. For further information regarding referrals and plans of care for specific domains, consult the associated modules.

Q1. **Patient:** What is your housing situation today?(1) **QM**

- Do not have housing (eg, staying with others, hotel, shelter), specify: _____ [B1] [P1] [P2]
Housing Insecurity  CCG
- Have own housing (eg, house, apartment), but afraid of losing it in the future [P1] [P2] Housing Insecurity  CCG
- Have own housing
- Other, specify: _____ [P1] [P2]
- Unknown

Q2. **Patient:** Do you have any problems with the place you live? (Check all that apply.) **QM**

- Appliances not working or unavailable (eg, stove, refrigerator) [P1] [P2] Housing Insecurity  CCG
- Bug infestation [P1] [P2] Housing Insecurity  CCG
- Electricity not working or unavailable [P1] [P2] Housing Insecurity  CCG
- Heating insufficient [P1] [P2] Housing Insecurity  CCG
- Lead paint or pipes [P1] [P2] Housing Insecurity  CCG
- Mold [P1] [P2] Housing Insecurity  CCG
- Smoke detectors not working or unavailable [P1] [P2] Housing Insecurity  CCG
- Water leaks [P1] [P2] Housing Insecurity  CCG
- Water not working or unavailable [P1] [P2] Housing Insecurity  CCG
- Other, specify: _____ [P1] [P2] Housing Insecurity  CCG
- None
- Unknown

Q3. **Patient:** In the past year, how often did you worry that you would run out of food before you got money to buy more? **QM**

- Often [B2] [P1] [P3] Food Insecurity  CCG
- Sometimes [B2] [P1] [P3] Food Insecurity  CCG
- Never
- Unknown

Q4. **Patient:** In the past year, how often did the food run out and you did not have money to buy more? **QM**

- Often [B2] [B3] [P1] [P3] Food Insecurity  CCG
- Sometimes [B2] [B3] [P1] [P3] Food Insecurity  CCG
- Never
- Unknown

Q5. Patient: In the past year, have you missed medical appointments, work, or getting things needed for daily living because you did not have transportation? (Check all that apply.) **QM**

- Yes, it has kept me from medical appointments or getting medicines. [B4] [P1] [P4] Transportation  CCG
- Yes, it has kept me from nonmedical activities, work, or getting things needed for daily living. [B4] [P1] [P4] Transportation  CCG
- Other, specify: _____ [B4] [P1] [P4] Transportation  CCG
- None
- Unknown

Q6. Patient: In the past year, has the electricity, water, gas, or oil company threatened to shut off services where you live?

- Yes, not shut off [P1] [P3]
- Yes, already shut off [B3] [P1] [P3]
- No
- Unknown

Q7. Patient: How often does anyone, including your family, physically hurt you?

- Never
- Rarely [P1] [P5] Intimate Partner Violence  CCG
- Sometimes [P1] [P5] Intimate Partner Violence  CCG
- Fairly often [P1] [P5] Intimate Partner Violence  CCG
- Frequently [P1] [P5] Intimate Partner Violence  CCG
- Unknown

Q8. Patient: How often does anyone, including your family, insult or talk down to you?

- Never
- Rarely [P1] [P5] Intimate Partner Violence  CCG
- Sometimes [P1] [P5] Intimate Partner Violence  CCG
- Fairly often [P1] [P5] Intimate Partner Violence  CCG
- Frequently [P1] [P5] Intimate Partner Violence  CCG
- Unknown

Q9. Patient: How often does anyone, including your family, threaten to harm you?

- Never
- Rarely [P1] [P5] Intimate Partner Violence  CCG
- Sometimes [P1] [P5] Intimate Partner Violence  CCG
- Fairly often [P1] [P5] Intimate Partner Violence  CCG
- Frequently [P1] [P5] Intimate Partner Violence  CCG
- Unknown

Q10. Patient: How often does anyone, including your family, scream or curse at you?

- Never
- Rarely [P1] [P5] Intimate Partner Violence  CCG
- Sometimes [P1] [P5] Intimate Partner Violence  CCG
- Fairly often [P1] [P5] Intimate Partner Violence  CCG
- Frequently [P1] [P5] Intimate Partner Violence  CCG
- Unknown

Q11. Patient: How hard is it for you to pay for the very basics, such as food, housing, medical care, and heating?

- Very hard [B3] [P1] [P3] Financial Status and Benefits  CCG
- Somewhat hard [B3] [P1] [P3] Financial Status and Benefits  CCG
- Not hard at all
- Unknown

Q12. Patient: Do you get the help you need with day-to-day activities, such as bathing, preparing meals, shopping, and managing finances?

- Yes
- No [P1] [P6] Caregiver Resources Evaluation  CCG Community Resources  CCG
- Unknown

Q13. Patient: Do you feel lonely or isolated from those around you?

- Yes [P1] [P6] Social Support  CCG
- No
- Unknown

Quality Measures

- HEDIS Measures include(2):
 - Social needs screening and intervention is completed.
-

Plan of Care

Barriers

- B1.** Housing inadequate
- B2.** Food insufficient
- B3.** Financial barrier
- B4.** Transportation inadequate

Problems

P1. Social determinant of health need present

- **Goal:** Social determinant of health needs assessed and addressed(3)(4)(5)(6) **NN**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **EDUCATE:** Assessment of social determinants of health is routinely completed on every person.(7)(8)(9)
 - The purpose is to identify resources that could be helpful. Information is still valuable to the program even if the person does not have any needs in a particular domain.
 - Availability of resources and interventions varies by region.
 - **EDUCATE: Resources:**
 - Benefit Finder: www.benefits.gov
 - Findhelp: www.findhelp.org
 - United Way: www.211.org
 - **COORDINATE:** Increased intervals of follow-up with preferred method of communication (eg, phone, text, email)
 - **COORDINATE:** Social worker referral, as needed(10)
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Follow-up contact to evaluate social needs status, referral status, and need for further assistance(11)(12)

P2. Housing inadequate, unstable, or unsafe

- **Goal:** Housing modified or changed so it is adequate, stable, and safe(13) **NN**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying housing options
 - **ASSIST:** Completing documentation for housing assistance, if needed
 - **ASSIST:** Appointment scheduling with provider if housing problems negatively affecting self-care
 - **COORDINATE:** Social worker referral, as needed(10)
 - **COORDINATE:** Community services referral, as needed(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate housing status, referral status, and need for further assistance(11)(12)

P3. Financial issues present

- **Goal:** Financial issues addressed(13)(16)(17)(18) **NN**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying financial support programs(19)
 - **ASSIST:** Contacting and completing paperwork for financial support programs

- **ASSIST:** Appointment scheduling
- **EDUCATE:** Healthcare benefits and out-of-pocket expenses
- **EDUCATE: Resources:**
 - Administration for Children & Families: www.acf.hhs.gov
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Food and Nutrition Service (FNS) Nutrition Programs: www.fns.usda.gov/programs
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds. Find help with the cost of medicine: www.needymeds.org
- **COORDINATE:** Financial assistance referral; examples include:
 - Area Agency on Aging
 - Disease advocacy
 - Financial counseling or assistance
 - Food banks or other sources for nutritional support and financial assistance
 - Government agencies
 - Legal aid
 - Long-term disability application
 - Medicaid application
 - Medicare application
 - Pharmacy assistance programs
 - Short-term disability application
 - Supplemental Security Income (SSI) or disability benefits application
 - Veterans Affairs (VA) benefits application
- **COORDINATE:** Care team meeting, if needed(20)(21)
- **COORDINATE:** Communication between care team members and care plan reconciliation(20)(22)(23)
- **COORDINATE:** Follow-up for evaluation of financial issues, referral status, and need for further assistance(11)(12)

P4. Transportation inadequate for patient's needs

- **Goal:** Transportation needs adequately met(13)(16)(24) **N**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying transportation resources and accessing as needed(25)
 - **ASSIST:** Completing application forms for disability transportation, if needed
 - **EDUCATE:** Type of transportation resources and how to access them
 - **EDUCATE:** When and how to get help if transportation continues to be a problem
 - **COORDINATE:** Social worker referral, as needed(10)
 - **COORDINATE:** Community services referral, as needed(26)
 - **COORDINATE:** Communication between care team members on how patient is transported to and from service visits(20)(22)(23)
 - **COORDINATE:** Follow-up contact to evaluate transportation, referral status, and need for further assistance(11)(12)

P5. Personal safety risk present

- **Goal:** Safety issues identified and addressed(13)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Contact with legal authorities, as appropriate(27)
 - **EDUCATE:** When and how to get help in unsafe situations
 - **EDUCATE: Resources:**
 - National Domestic Violence Hotline: www.thehotline.org; 1-800-799-7233, or call, text, or live chat 88788
 - National Suicide Prevention Lifeline: www.988lifeline.org; 1-800-273-8255, or call, text, or live chat 988
 - **COORDINATE:** Protective services or legal authorities, as needed
 - **COORDINATE:** Social worker referral, as needed(10)
 - **COORDINATE:** Home care referral, if indicated(27)
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Follow-up contact to evaluate safety, referral status, and need for further assistance(11)(12)

P6. Social support needs present

- **Goal:** Social support needs identified and addressed

o Interventions:

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Identifying caregivers and community resources
- **ASSIST:** Accessing caregivers and community resources, if needed
- **ASSIST:** Completing application forms for community resources, if needed
- **EDUCATE:** When and how to get help if social support continues to be a problem
- **COORDINATE:** Social worker referral, as needed(10)
- **COORDINATE:** Home care referral, if indicated(27)
- **COORDINATE:** Follow-up contact to evaluate social support, referral status, and need for further assistance(11)(12)

Evidence Summary

Quality standards: Standards indicate that program content should address psychosocial issues that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(28)

Evidence: A review of 4 independent studies with 14,749 low-income adults indicated that participants who were less healthy, had lower incomes, and had higher levels of stress and depressive symptoms were consistently more likely to have multiple social needs than those who were healthier and had higher incomes, lower stress, and fewer depressive symptoms. Women with children were less likely to need food, transportation, or money for necessities. Older, less healthy adults needed food, transportation, and money for necessities and unexpected expenses.(29)

Quality standards: Standards indicate that program content should address psychosocial issues (eg, inadequate or unsafe housing) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(28)

Evidence: A retrospective review of 22,324 medical records determined that 3 unstable housing circumstances (behind on rent in the past 12 months, 2 or more moves in the past 12 months, and a history of homelessness in the child's lifetime) are associated with adverse caregiver and child health status, developmental risk, and other household hardships.(30)

Quality standards: Standards indicate that program content should address psychosocial issues (eg, financial barriers) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(28)

Evidence: A retrospective study of 7959 nationally representative US children determined that household food insecurity is associated with higher emergency department use, higher school absenteeism, and lower access to care.(31)

Quality standards: Standards indicate that program content should address psychosocial issues (eg, transportation) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(28)

References

1. Billioux A, Verlander K, Anthony S, Alley D. Standardized Screening for Health-Related Social Needs in Clinical Settings: the Accountable Health Communities Screening Tool. [Internet] National Academy of Sciences. 2017 May Accessed at: <https://nam.edu/>. [accessed 2023 Sep 18] [Context Link 1]
2. HEDIS quality measures 2023. [Internet] NCQA. Accessed at: <https://www.ncqa.org/hedis/measures/>. Updated 2023 [accessed 2023 Oct 23] [Context Link 1]
3. Eder M, et al. Screening and interventions for social risk factors: technical brief to support the US Preventive Services Task Force. Journal of the American Medical Association 2021;326(14):1416-1428. DOI: 10.1001/jama.2021.12825. [Context Link 1] View abstract...
4. Daniel H, Bornstein SS, Kane GC, Health and Public Policy Committee of the American College of Physicians. Addressing social determinants to improve patient care and promote health equity: an American College of Physicians position paper. Annals of Internal Medicine 2018;168(8):577-578. DOI: 10.7326/M17-2441. [Context Link 1] View abstract...
5. Ruiz Escobar E, Pathak S, Blanchard CM. Screening and referral care delivery services and unmet health-related social needs: a systematic review. Preventing Chronic Disease 2021;18:Online. DOI: 10.5888/pcd18.200569. [Context Link 1] View abstract...
6. Health Equity Plus Standards and Guidelines. Effective for Surveys Beginning on or After July 1, 2024 [Internet] National Committee for Quality Assurance (NCQA). 2024 Accessed at: To purchase copies of this publication, visit <https://ncqa.org>. [accessed 2023 Sep 05] [Context Link 1]
7. A Guide to Using the Accountable Health Communities Health-Related Social Needs Screening Tool. Promising Practices and Key Insights [Internet] Centers for Medicare & Medicaid Services. 2022 Aug Accessed at: <https://www.cms.gov>. [accessed 2023 Aug 15] [Context Link 1]
8. Ezell JM, Hamdi S, Borrero N. Approaches to addressing nonmedical services and care coordination needs for older adults. Research on Aging 2022 Mar-Apr;44(3-4):323-333. DOI: 10.1177/01640275211033929. [Context Link 1] View abstract...
9. Tuzzio L, et al. Social risk factors and desire for assistance among patients receiving subsidized health care insurance in a US-based integrated delivery system. Annals of Family Medicine 2022 Mar-Apr;20(2):137-144. DOI: 10.1370/afm.2774. [Context Link 1] View abstract...

10. McQueen A, et al. Social needs, chronic conditions, and health care utilization among Medicaid beneficiaries. *Population Health Management* 2021;24(6):681-690. DOI: 10.1089/pop.2021.0065. [Context Link 1, 2, 3, 4, 5] View abstract...
11. Care coordination and care transitions to support adherence. In: Perez R, Rogers S, Prince M, Fraser K, editors. *Case Management Adherence Guidelines*. 2020 ed. Case Management Society of America; 2022:43-56. [Context Link 1, 2, 3, 4, 5, 6]
12. Monitoring and evaluation. In: Gillingham DC, editor. *Foundations of Case Management*. Blue Bayou Press; 2021:51-52. [Context Link 1, 2, 3, 4, 5, 6]
13. Davidson KW, et al. Developing primary care-based recommendations for social determinants of health: methods of the U.S. Preventive Services Task Force. *Annals of Internal Medicine* 2020;173(6):461-467. DOI: 10.7326/M20-0730. [Context Link 1, 2, 3, 4] View abstract...
14. Wormley K, Dickson D, Alter H, Njoku N, Imani P, Anderson ES. Association of social needs and housing status among urban emergency department patients. *Western Journal of Emergency Medicine* 2022;23(6):802-810. DOI: 10.5811/westjem.2022.8.55705. [Context Link 1] View abstract...
15. Hadenfeldt C, Todd MJ, Hamzhie C. Medical respite post-hospitalization for adults experiencing homelessness. *Nursing* 2023;53(3):47-52. DOI: 10.1097/01.NURSE.0000918524.41501.98. [Context Link 1] View abstract...
16. Health care delivery. In: Harding M, Hagler D, editors. *Conceptual Nursing Care Planning*. Elsevier; 2022:320-338. [Context Link 1, 2]
17. Rector C. Structure and economics of community/public health services. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:121-169. [Context Link 1]
18. Addressing social determinants: clinical versus nonclinical issues. In: Fraser K, Perez R, Latour C, editors. *CMSA's Integrated Case Management*. New York, NY: Springer Publishing Company LLC; 2018:139-148. [Context Link 1]
19. Morris BC. Patient and caregiver teaching. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:49-62. [Context Link 1]
20. Case management standards and professional organizations. In: Powell SK, Tahan H, editors. *Case Management a Practical Guide for Education and Practice*. 4th ed. Philadelphia, PA: Wolters Kluwer, Lippincott, Williams & Wilkins; 2019:314-354. [Context Link 1, 2, 3]
21. Ralston JD, Wagner EH. Comprehensive chronic disease management. In: Goldman L, Schafer AI, editors. *Goldman-Cecil Medicine*. 26th ed. Elsevier; 2020:42-45.e2. [Context Link 1]
22. Karam M, et al. Nursing care coordination for patients with complex needs in primary healthcare: a scoping review. *International Journal of Integrated Care* 2021;21(1):16. DOI: 10.5334/ijic.5518. [Context Link 1, 2] View abstract...
23. Joo JY, Liu MF. The experience of chronic illness transitional care: a qualitative systematic review. *Clinical Nursing Research* 2022;31(2):163-173. DOI: 10.1177/10547738211056166. [Context Link 1, 2] View abstract...
24. Sutter RE, Hernandez L. Working with vulnerable people. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:667-688. [Context Link 1]
25. Scanlon A, Reinisch C. Social determinants of health. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:18-20. [Context Link 1]
26. Bharmal N. Social determinants and health equity in functional medicine. *Physical Medicine and Rehabilitation Clinics* 2022;33(3):665-678. DOI: 10.1016/j.pmr.2022.04.007. [Context Link 1] View abstract...
27. Greenberg SA. Chronic illness and older adults. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:63-80. [Context Link 1, 2, 3]
28. Standards of Practice for Case Management. [Internet] Case Management Society of America. 2022 Accessed at: <https://www.cmsa.org/>. [accessed 2023 Sep 21] [Context Link 1, 2, 3, 4]
29. Kreuter MW, et al. How do social needs cluster among low-income individuals? *Population Health Management* 2021;24(3):322-332. DOI: 10.1089/pop.2020.0107. [Context Link 1] View abstract...
30. Sandel M, et al. Unstable housing and caregiver and child health in renter families. *Pediatrics* 2018;142(2):e20172199. DOI: 10.1542/peds.2017-2199. [Context Link 1] View abstract...
31. Peltz A, Garg A. Food insecurity and health care use. *Pediatrics* 2019;144(4):Online. DOI: 10.1542/peds.2019-0347. [Context Link 1] View abstract...

Codes

ICD-10 Diagnosis: Z56.0, Z56.1, Z56.81, Z56.89, Z56.9, Z58.6, Z59.00, Z59.01, Z59.02, Z59.2, Z59.3, Z59.41, Z59.48, Z59.5, Z59.6, Z59.7, Z59.811, Z59.812, Z59.819, Z59.87, Z59.89, Z59.9, Z65.4, Z65.5, Z65.8, Z65.9, Z71.88 [Hide]

MCG Health
Chronic Care 28th Edition
Copyright © 2024 MCG Health, LLC
All Rights Reserved

Last Update: 3/14/2024 3:13:31 AM
Build Number: 28.1.20240313235330.016020